Louisiana Treatment Venture

Capital Raising Plan

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Status: Financing analysis — companion to the capital requirements plan

Scope: Financing analysis, companion to the capital requirements plan of the same date. Not legal, tax, securities, or investment advice. Securities and healthcare-regulatory structures named here require counsel before use. Figures are tagged by provenance; broker and lender marketing content is marked as directional only. Section 8 lists every item requiring independent confirmation.

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Louisiana Treatment Venture — Capital Raising Plan

Prepared: 2026-10-06 · Companion to the capital-requirements plan of the same date.

The requirement, from the companion document: $150–255K (recovery residences), **$400–700K (licensed ASAM 2.1 IOP)**, $900K–2.0M (ASAM 3.5 residential). Roughly 60% of the IOP figure is pre-revenue payroll through licensure and credentialing.

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0. Executive summary

0.1 Reliability comes from sequencing and stack design, not from finding the right source

There is no single source that funds this. The financeable structure is a **stack**: personal cash

- SBA 7(a) + a full-standby seller note + a small accredited friends-and-family raise, with **non-dilutive public money entering as revenue, not capital**, and a Louisiana state program used specifically to keep a lien off your house.

0.2 The asymmetry that should drive the whole plan

SBA loan-level default data, FY2020–2025 [FOIA]:

Loan purpose	Default rate
Change of ownership (acquisition)	**0.71%**
Existing business, >2 years	1.20%
Startup, funds will open business	1.43%
New business / ≤2 years	**1.99%**

Acquisitions default roughly **3× less** than new-business loans, and lenders price and approve accordingly. **A de novo IOP is the single hardest thing in this plan to debt-finance**, and a rule change five days ago made it harder: **SOP 50 10 8.1, effective 10/1/2026**, moved acquisition underwriting to a new Appendix 15 requiring **1.25× DSCR — with projections no longer counting toward the floor** [SOP]. A pre-revenue IOP has nothing *but* projections. Portfolio-wide 7(a) defaults hit **4.8% in March 2026**, the highest since 2013; lenders are tightening.

Plan to fund a de novo IOP mostly with equity, or buy instead of build.

0.3 The one number that governs your personal downside

$500,000. Above that, an SBA loan must be fully collateralized where possible — and if business collateral is insufficient, **the lender must take a lien on personal real estate, including a second mortgage on a home with available equity.** The personal guarantee itself is **uncapped**, payable on demand, and the lender need not exhaust business collateral first.

A bare guarantee produces a judgment that may not reach a protected homestead. **A recorded mortgage is foreclosable.** Louisiana's homestead exemption does not protect against a mortgage you consensually granted. After foreclosure the deficiency survives, SBA acquires it, and it can garnish wages. **Chapter 7 discharges personal liability but does not strip a validly recorded lien.**

The specific mitigation: Louisiana LED's **Collateral Support** program pledges **cash up to $250,000** to secure loans up to $1M [LED]. That is the cleanest available lever for keeping a mortgage off your home, and it should be arranged *before* a lender structures collateral, not after.

0.4 Three things the individual research threads each missed

(a) Rural siting unlocks three separate doors at once. LDH's **Rural Health Transformation Program** names "rural behavioral health or substance use providers" as explicitly eligible; **USDA B&I guarantees 85%** on loans under $5M and is commonly used for rural healthcare facilities, with wide Louisiana eligibility; and **LED Quality Jobs**, which otherwise excludes medical professionals, has a qualifying path for siting in a parish in the **lowest 25% by per capita income**. Site selection is therefore a financing decision, not just a market decision.

(b) Opioid settlement money fixes a payer-mix problem, not a capital problem. Parishes must prioritize people **not covered by Medicaid or private insurance** — exactly the population a Medicaid-only pro forma excludes, and exactly the concentration risk the companion plan flags as High. It arrives as a service contract, so it diversifies revenue rather than funding build-out.

(c) Louisiana's corporate-practice posture is softer than the companion plan implied. The LSBME's **December 2024 Statement of Position** holds that physician employment by a non-professional corporation is **not *per se* unlawful**; the operative test is whether the arrangement impairs independent medical judgment. For a non-clinician owner this materially simplifies structuring versus strict-CPOM states.

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1. Debt

1.1 SBA 7(a) — current terms

Item	Current
Maximum	$5M, **plus** $5M of 504 (decoupled since July 2026)
Guarantee	75% above $150K
Rate	Variable cap base **+3.0%** over $350K; well-qualified borrowers ~**8.5–9.25%**; FY2025 Q4 acquisition loans averaged **8.86%**
Guarantee fee (FY2026)	3% on $150–700K; 3.5% to $1M; 3.75% above
Amortization	**10 years** business portion; 25 years with real estate
Equity injection	**10% minimum**; acquisitions require **≥5% cash**; lenders routinely want 15–20%, **20%+ for true startups**

Approval rates run ~67% for 7(a) versus ~43% conventional [vendor].

Correction to my earlier note: I wrote that seller notes counted toward equity injection require 24 months of deferral. **That rule is obsolete.** Under SOP 50 10 8 (eff. 6/1/2025) and carried into 8.1, a seller note counts toward the injection **only on full standby — no principal and no interest — for the entire life of the SBA loan**, and may not exceed 50% of the injection.

1.2 The credentialing gap cannot be factored

This is the most important negative finding in the research. Medical receivables factoring advances against **existing billable claims**. During licensure and credentialing you have no claims at all — there is nothing to factor. Factoring addresses the 30–90 day claims-aging lag, not the 6–12 month pre-revenue window.

Even post-credentialing it is unattractive to a first-time biller: expect **3–5% per 30 days at 60–75% advance**, not advertised rates; most factors require **6–12 months of operating history and $10–20K/month volume**; and **Medicaid receivables cannot be directly assigned**, requiring sweep-account structures, with Medicaid advances at 65–80% versus commercial 75–90% [vendor].

Therefore: the $250–400K credentialing reserve must be equity or 7(a) working capital funded at close. Do not plan to draw it later.

1.3 SBA 504 and other debt

504 finances owner-occupied real estate at roughly 50% bank / 40% CDC debenture / **10% borrower down** — but **15% for single-purpose property**, which residential treatment may be classified as. It **cannot fund working capital**; pair with 7(a), which must be approved first.

Louisiana and federal programs worth using:

- **LED Collateral Support** — pledged cash to $250K securing loans to $1M. *The home-lien lever.*
- **LED Small Business Loan Guaranty** — up to 80% or $1.5M, 15% minimum equity, fee 0–2%. New rule dated 5/20/26; pull current text.
- **USDA B&I** — 85% guarantee under $5M, commonly used for rural healthcare.
- **SSBCI** — up to $113M deployed through LEDC via lenders, not direct.
- **CDFIs serving Louisiana** — DreamSpring ($1K–350K), LiftFund ($500–$1M), TruFund ($500–$1M), LISC.

1.4 Lender selection matters more than borrowers assume

No SBA lender markets a behavioral health vertical. Live Oak is #1 by dollar volume ($2.68B CY2025, average $1.25M) with dedicated healthcare, veterinary and dental teams — but behavioral health is **not** among them, and vertical expertise is precisely Live Oak's edge. Huntington is #1 by loan count (6,071 loans, average $307K, 10% down, fee waivers for underserved borrowers). Portfolio default rates vary widely by lender — **1.90% (Huntington) to 6.28% (First Internet Bank)** — which is a proxy for underwriting posture and workout behavior.

Screen for PLP status. A non-PLP lender adds 2–4 weeks of SBA review, which can expire an LOI's exclusivity window.

On sector stigma: **no formal SBA industry exclusion exists** — substance abuse treatment centers are explicitly permitted. Reported lender reluctance (regulatory complexity, reimbursement uncertainty, staffing

costs) comes **entirely from lender and broker marketing content**; no primary bank policy document was found. Treat as unverified.

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2. Non-dilutive public money — revenue, not capital

Bottom line: no non-dilutive source will fund your build-out as a for-profit startup. Every meaningful pool is nonprofit-restricted, state-intermediated, too large-scale, or structured as a service contract. Three are still worth real effort.

2.1 Parish opioid settlement funds — the best target

Louisiana's State-Local MOU created the **Opioid Abatement Task Force**, with administration delegated to the **Opioid Abatement Administration Corporation**. The split is **80% local governments / 20% sheriffs**, and **parishes and sheriffs each decide autonomously** — there is no statewide application.

The money: ~$600M to parishes and sheriffs across CY2022–2038. As of October 2024 approximately **$98.5M had been distributed but only ~$8.6M actually spent** by 20 parishes and 24 sheriffs. Those unspent balances are your leverage.

The published model — Calcasieu Parish: 30-day prequalification screen → full application; **up to $500,000 in year one**; annual payouts projected $600–800K; 12 entities funded in round one; **reimbursement-based**.

For-profit eligibility: not categorically barred — Calcasieu's language covers "organizations, agencies, and service providers." Two constraints bind: parishes must **prioritize treatment for people not covered by Medicaid or private insurance**, and the practical vehicle is **contracting to deliver services or buy beds**, not receiving startup capital.

Verdict: accessible as a revenue contract. Approach individual parishes. Reimbursement-based means you must be able to float the cost first — which is itself a working-capital requirement.

2.2 State Opioid Response — a named slot if you offer MOUD

LDH's **Office of Behavioral Health** is the grantee at **$18,001,460 (FY2025)**, flowing to **10 Local Governing Entities** covering all 64 parishes. Louisiana runs a hub-and-spoke model: **10 OTP hubs plus 50 office-based opioid treatment (OBOT) spokes — five per LGE — with financial support provided to participating OBOT providers**, plus a Spoke Care Team. Volume expectation is 20 new patients in year one, 10 in year two.

Verdict: plausibly accessible as a subrecipient if your IOP includes medication for opioid use disorder. **Contact your LGE, not LDH.** This is the most concrete named slot in the state for a new clinical provider — and adding MOUD is a program-design decision you can make now.

2.3 Rural Health Transformation Program — if you site rurally

LDH's **Rural Collaborative Provider Models Program** explicitly names "rural behavioral health or substance use providers" as eligible. A separate **Capital Improvement** round ran **$100,000–$10M with a 10% cash/in-kind match** (applications closed 7/10/2026; Year-1 funds must be spent by 9/30/2027). Note the

Collaborative Models notice **excludes capital improvements, equipment, telehealth infrastructure, and EHR modernization**.

Verdict: conditionally accessible — and it should influence site selection. Monitor the RHTP funding-opportunity page for new windows.

2.4 Ruled out

- **NMTC** — most transactions require $2–5M minimum; projects under $7M are unlikely to secure allocation; average healthcare NMTC project is **$11.2M**. Your project is too small.
- **CCBHC** — Louisiana joined the Section 223 demonstration **5/28/2026** with staggered start dates through 7/1/2027, and cost-based PPS economics materially beat Medicaid FFS. But it requires **nine services with at least three delivered directly**, and eligible entities are typically CMHCs, FQHCs, state-designated providers and nonprofits. **Not reachable by a new single-service IOP.** Track as a 5-year aspiration or partnership target.
- **HUD Recovery Housing Program** — nonprofit acquisition/rehab only, administered through OCD-LGA. Not accessible to a for-profit.
- **LED Enterprise Zone** — sunset 6/30/2025.
- **Direct SAMHSA grants** — SOR and SABG flow to the state, which subawards; many programs are nonprofit-restricted.

Correction to the companion plan: I wrote that the FY26 budget proposed dissolving SAMHSA with ~$1B in cuts. **Congress rejected it** — appropriators refused the plan (an Appropriations Committee vote went **26–3 against, with funding added**) and approved a **$7.1B SAMHSA topline**; the Administration for a Healthy America has never been authorized or funded. However, HHS has proceeded administratively via staffing cuts and transferred duties, and the **FY27 request renews the proposal.** Net: the agency survives on paper and is weakened in practice.

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3. Equity, seller financing, and partnership

3.1 Self-funded search is the only open equity architecture

Traditional funded search targets **$2–10M EBITDA**; **self-funded search targets $500K–$3M** — your range. Self-funded searchers retain **50–80%** and draw cash from year one, with a typical stack of roughly **75% SBA / 15% seller note / 10% investor equity** at ~20% common plus a 10% preferred return.

Existence proof that ETA touches this sector: ETA Equity's portfolio includes **Windmill Wellness Ranch**, a co-occurring inpatient/outpatient SUD treatment center acquired in 2024.

Closed at your scale: traditional funded search funds, institutional PE, family offices, and health-system joint ventures. The binding reason is not capital scarcity — it is that **Louisiana's Medicaid-weighted payer mix caps entry *and* exit multiples at 3–5×**, which makes targets cheaper to buy and simultaneously makes outside equity uninterested in funding the purchase.

3.2 Seller financing — smaller and harder than LOI chatter implies

Broker survey data puts seller financing at **4–14% of deal value**, earnouts at 1–6%, and cash at close at **75–89%** [vendor].

The catch that kills most full-standby notes: a note on full standby for the life of the loan costs the seller roughly **61% of face value in present-value terms**, versus ~5% for a normal seller note. Sellers with competing offers routinely refuse. Negotiate standby treatment **before** the LOI binds.

Equity rollover is largely incompatible with SBA. If the seller retains **≥20%**, they must provide a **full unconditional personal guaranty for the life of the SBA loan**. Below 20% it is a limited guaranty. Also new under SOP 8.1: **seller earnouts are prohibited**, a quality-of-earnings report is required at purchase prices ≥$3M, and total debt is capped at appraised business value.

A Louisiana-specific leverage point [infer]: because a BHSP license does not transfer and you must re-license via CHOW, you carry genuine post-close regulatory risk that the seller controls. That is a legitimate basis to condition a portion of consideration — a seller note or holdback — on **LDH issuing the license to you**. Earnouts being prohibited under SOP 8.1 pushes this toward a standby note or escrow structure. This is a deal-structure question for counsel.

3.3 Partnering with a clinician — structure it as two separate things

Louisiana's softer CPOM posture (§0.4c) helps, but the anti-kickback constraint does not relax. Medical director compensation must be **fair market value, fixed and predictable, and decoupled from referral volume or value**. DOJ settled for **$5.3M in October 2024** over medical director fees paid without genuine services rendered.

Avoid "equity instead of salary." The clean structure is two separable items: (a) an hourly or fixed FMV stipend for **defined, logged duties** — build and keep the supervisory ledger; and (b) equity, if any, as a **separate arm's-length investment at FMV with real capital contributed** and proportional returns. Commercial reasonableness also constrains size: a thin-revenue startup cannot defend a large stipend.

Exposure note: Stark and AKS bite hardest on federal program business. A Medicaid-heavy Louisiana program is fully exposed; a cash-pay or commercial-only program materially less so, though state fee-splitting and CPOM rules still apply.

3.4 Friends and family — the right instrument

For an operating LLC with no priced round coming, **sell membership units under Reg D Rule 506(b) to accredited investors only**: PPM, subscription agreement, accreditation questionnaire, Form D. 506(b) preempts state blue-sky registration.

Two traps: **convertible notes are the wrong instrument** here — they exist to defer valuation to a future priced round that will never happen; and **admitting non-accredited investors** triggers registered-offering-grade disclosure that makes a small raise uneconomic, and a non-accredited convertible holder can destroy a later exemption. Note also that **raising search capital is itself a securities offering** subject to the same compliance.

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4. The recommended capital stack

4.1 If acquiring a licensed outpatient program (the financeable path)

Layer	Share	Notes
Personal cash	**5–10%**	≥5% must be genuine cash [SOP]
SBA 7(a)	~70–80%	10-yr amortization, ~8.5–9.25%
Seller note, full standby	up to 5% of injection	Costs seller ~61% of face in PV — expect resistance
Accredited F&F, Reg D 506(b) LLC units	balance	Holdco/MSO level
LED Collateral Support	up to $250K pledged	**Keeps the lien off your house**
Opioid settlement service contract	—	Revenue, post-close

4.2 If building a de novo IOP

Expect to fund **most of $400–700K with equity**, because projections no longer satisfy DSCR and a pre-revenue licensed facility has no cash flow to underwrite. Realistic composition: personal cash

- accredited F&F units + a modest 7(a) working-capital piece if a lender will take it. **This is the expensive path, and the research says so plainly.**

4.3 Structuring to stay under the $500K lien threshold

A first deal sized so that **SBA debt stays below $500,000** avoids the mandatory full-collateralization rule that reaches personal real estate. Combined with LED Collateral Support, this is the difference between "lose the invested capital" and "lose the capital, owe an uncapped guarantee, and face foreclosure on your home." **Size the first deal to the downside you can survive, not to the upside you want.**

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5. The sequenced plan

Phase 0 — Months 0–6: become financeable (costs nothing but time)

1. **Take the paid industry role** recommended in the companion plan. It is *also* the financing-optimal move: SBA dropped "experience and depth of management" as a regulatory criterion in 2023 [CFR 120.150, 88 FR 21 085], but **lenders retained it as credit policy**, and in licensed healthcare they commonly require relevant management experience plus evidence licensure will be reissued.
2. **Clean up personal credit and document everything.** Lenders assess credit, cashflow, and collateral — the three remaining §120.150 criteria.
3. **Get LDH's CHOW position in writing** before a lender asks. Lenders want evidence licensure will be reissued; Louisiana's rule is that it does not transfer. Resolve this on paper early.
4. **Open the parish conversation.** Identify your parish's opioid abatement committee and application cycle. Calcasieu's is published; others vary.
5. **Contact your LGE** about the OBOT spoke program if MOUD is in your program design.

Phase 1 — Months 6–12: assemble the package

6. **Engage a Louisiana healthcare-regulatory attorney and a securities attorney.** Different people. The first handles CHOW, CPOM, AKS/EKRA and medical-director structure; the second handles the Reg D offering.
7. **Build the lender package.** SBA Form 1919 (all owners ≥20%), Form 413 personal financial statement, Form 912 if required; 2–3 years business returns, YTD P&L and balance sheet, AR/AP aging, full debt schedule, 6 months bank statements, formation documents, licenses; per owner, 2 years personal returns, PFS, **résumé**, ID; business plan with detailed use of funds and 3-year projections. Acquisitions add LOI, purchase agreement, seller's 3-year financials, and a **certified valuation**.
8. **Screen lenders for PLP status and healthcare appetite.** Ask directly whether they have underwritten behavioral health, and what they require on licensure.
9. **Apply for LED Collateral Support** before collateral is structured.

Phase 2 — Months 12–18: execute

10. **LOI with an explicit financing contingency** and enough runway — 7(a) runs **60–90 days application to funding**, LOI to close **60–90 days** (60–120 for search-style deals). Full search-to-close commonly runs **6–12 months**; BizBuySell's 2025 median time-to-close was **170 days**.
11. **Negotiate seller-note standby treatment before the LOI binds.**
12. **Confirm 10+ years of remaining lease term** — SBA requires it, and a landlord who will not extend kills the deal.
13. **Condition closing on LDH CHOW approval**, or escrow against it.
14. **Fund the credentialing reserve at close.** It cannot be factored later.

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6. Deal-killers

Trap	Why it kills
Licensing transfer at close	Named as a top source of SBA closing delay generally — and in Louisiana the license does not transfer at all
<10 years lease term	SBA requirement; landlord refusal ends the deal
Seller refuses full standby	Costs them ~61% of face value in PV; most refuse when they have options
Valuation below negotiated price	Required above $250K; the gap must be closed in buyer cash
Non-PLP lender	Adds 2–4 weeks; can expire LOI exclusivity
Seller retains ≥20%	Triggers full unconditional personal guaranty from the seller for the life of the loan
Target under revocation/provisional licensure	**Cannot undergo a CHOW at all** — the distressed bargains are often legally unsellable
Non-accredited F&F investor	Destroys the Reg D exemption; makes the raise uneconomic
Planning to factor the credentialing gap	Nothing to factor — no claims exist yet

7. Corrections to prior documents

15. **Seller-note standby is the life of the loan, not 24 months.** My earlier note said 24 months; that rule is obsolete under SOP 50 10 8 / 8.1.
16. **SAMHSA was not dissolved.** Congress rejected it 26–3 and added funding ($7.1B topline). HHS proceeded administratively; FY27 renews the proposal.
17. **Louisiana CPOM is softer than the companion plan implied** — LSBME's December 2024 Statement of Position holds physician employment by a non-professional corporation is not *per se* unlawful.
18. **SBA does not gate on industry experience** (13 CFR §120.150 as amended 2023) — but lenders do, as credit policy. The practical effect is the same; the mechanism matters when arguing with a lender.

8. Verify before relying

- **SOP 50 10 8.1 took effect 10/1/2026 and applies by loan-number date, not application date.** Confirm which SOP governs any deal in flight.
- **LED program terms** — the Small Business Loan Guaranty rule is dated 5/20/26; pull current text. Confirm Collateral Support availability and caps directly.
- **Parish opioid settlement eligibility rules outside Calcasieu were not found.** Each parish decides autonomously; do not generalize from Calcasieu.
- **Louisiana's selected CCBHC PPS methodology and provisional rates were not found.**
- **Factoring rates, lender reluctance toward behavioral health, and seller-financing percentages** all come from broker/lender marketing. Directional only; no primary source found.
- **Securities structures** (506(b), PPM contents, accreditation verification) require counsel.
- **The CHOW-at-close structuring question** — escrow, holdback, closing condition, or interim management agreement — is a legal question I could not resolve from public sources.
- **Texas remains unanalyzed.** None of the Louisiana-specific programs here transfer.

9. Bottom line

The capital is raisable, but the structure matters more than the amount.

19. **Buy rather than build if you can.** 0.71% versus 1.99% default rates is why lenders say yes, and SOP 8.1's removal of projections from DSCR math makes a de novo IOP substantially an equity-funded proposition.
20. **Keep SBA debt under $500,000 on the first deal, and use LED Collateral Support.** This is the difference between a recoverable failure and foreclosure on your home.
21. **Treat opioid settlement contracts and the SOR OBOT spoke as revenue strategy, not fundraising.** They solve the Medicaid-concentration risk the companion plan rates High — and the settlement money targets precisely the uninsured population a Medicaid pro forma ignores.

22. **Let site selection carry financing weight.** Rural siting opens RHTP, USDA B&I, and the LED Quality Jobs low-income-parish path simultaneously.
23. **The paid industry role is the cheapest financing move available.** It builds the management record lenders require, at positive income rather than negative.

The honest frame carried from the companion plan still holds: against a ~$145K baseline, this path spends years and real capital to reach comparable income, with an uncapped personal guarantee added. The financing research does not change that — it only shows which version of the bet is survivable if it goes wrong.